



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : رئيسه فهد ابراهيم وزير
المحافظة : سوهاج
الحى / المدينة : البليتا
رقم البطاقة : 26204012600626 رقم التليفون : 01115286114
العنوان : البليتا
العمر (تاريخ الميلاد) : 01-04-1962 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 30-09-2026

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (.....)
المقر بما فيه
التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالخلايا البلازميه
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالخلايا البلازميه
خطة العلاج : أورام كيماوي أبحاث وعينه [10853] قسم العلاج : الاورام
التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة:
الوظيفة:
الوظيفة:
التوقيع:
التوقيع:
التوقيع:
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / د. / د. / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: رئيسه فهم ابراهيم وزير
National ID: 26204012600626



تقرير طبي

السيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وماعة الخروج :
PM 01:17 05-07-2026

تاريخ وماعة الدخول :
--:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

A patient of plasma cell cancer for Clinical research and specimen

الوقت :

--:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين



جمهورية مصر العربية
مطابق لوائح الشخصية

رئيسه
فهد ابراهيم وزيرى
القنصلية
مركز البليفا - سوهاج

KC8193988

٢٦٢٠٤٠١٢٦٠٠٦٢٦



جمهورية مصر العربية
بطاقة تحقيق الشخصية

رئيسه
فهم ابراهيم وزيرى
التقييمه
مركز البليتا - سوهاج

١٩٩٦/٠٤/٢١

٢٢٢٠٤٠١٢٦٠٠٦٢٦

KC8193988



٢٠٣٣/٠٩

٢٢٢٠٤٠١٢٦٠٠٦٢٦

لا يغير



٢٠٣٣/٠٩

لا يغير

المنطقة مسرية حتى ٢٠٣٣/٠٩/٠٩



١٩٩٦

PAT. NAME: RAAESA FAHIM IBRAHIM
DATE: 11/03/2025

PROF. DR. SAMER SHEHATA
PAT. AGE: 63 YEAR(S)

MRI OF THE WHOLE BODY

THE BRAIN MRI SHOWS:

- Multiple small foci of hyperintense T2 signal periventricular with no mass effect .
- No intracranial space-occupying lesions , no mass effect .
- Normal appearance of the ventricular system , and the peri-hemisph. C.S.F. spaces .
- normal midline structures , no midline shift
- no pathological calcification
- Normal posterior fossa .

THE NECK MRI SHOWS:

- No enlarged cervical lymph nodes both sides .
- Normal size , and shape of the thyroid gland .
- Normal appearance of the cervical great vessels .
- Centralized trachea , and esophagus .
- Normal appearance of the pharynx , and the larynx .

THE CHEST MRI SHOWS:

- No sizable pulmonary nodules, masses, areas of consolidation or cavity lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal appearance of the thoracic aorta and the great vessels.
- Normal appearance of the chest wall.

THE ABDOMEN & PELVIS MRI SHOWS:

- Liver is average size, of homogenous texture and smooth outline . No focal lesions no intra hepatic biliary channel dilatation . normal caliber of the portal vein with no evident thrombosis
- Normal size , and shape of the pancreas , no focal masses

CENTER

PAT. NAME: RAAESA FAHIM IBRAHIM
DATE: 11/03/2025

PROF. DR. SAMER SHEHATA
PAT. AGE: 63 YEAR(S)

- Normal sized spleen of homogenous texture with no focal lesions.
- Normal appearance of the supra-renal gland both sides , no cystic or solid mass lesions .
- Both kidneys show normal appearance , with normal parenchymal thickness , and dye excretion . No dilated P.C.S. .
- Normal L/B with no stones or masses
- Normal uterus and adnexae
- No enlarged abdominal or pelvic lymph nodes .
- No fluid collection in the peritoneal cavities

THE EXAMINED PARTS OF THE SKELETON SHOWS / SKULL, BONY CHEST WALL, WHOLE SPINE AND BONY PELVIS:

- Heterogeneous abnormal signal examined bones esp. the spine vertebrae and the left iliac bone with no soft tissue component.
- There is decrease D11 and L2 vertebral height with mild posterior displacement indenting the anterior epidural fat.
- Normal para spinal muscles.

CONCLUSION:

- HISTORY OF BREAST CANCER, THE CURRENT STUDY SHOWS MULTIPLE BONY METASTASES PROMINENT AT LEFT ILIAC BONE AND DECREASE D11 AND L2 VERTEBRAL HEIGHT WITH MILD POSTERIOR DIOSPLACEMENT.
- SEMINLE BRAIN ATROPHY WITH SMALL VESSELS DISEASE.
- NO DETECTED LUNG NODULES.
- NO HEPATIC FOCAL LESIONS.
- NO ENLARGED LYMPH NODES.

Sincerely,

DR. TAREM M. MANSOUR, M.D.



NAME: MRS. RAJEISA FAHIM IBRAHIM . SEX: F.

AGE: 63Y.

EXAMINATION: BILAT. SONO-MAMMOGRAPHY.

DATE: 11/03/2025.

REF. DR. : PROF. DR. SAMIR SHELATAI

Report

- Rt. U.L.Q. small hypoechoic mass lesion, of irregular spiculated outline, measures about 6mm. diam, shows skin retraction, located at 11 O.Clock about 5cm. away from the nipple..
- Normal Lt. breast sonography ..
- No dilated retro-areolar ducts both sides.
- No significant enlarged bilateral axillary L. nodes.
- The mammography examination shows :

Small mass shadow at Rt. upper lateral para-areolar with peripheral Tu. Spiculae . No micro-calcifications detected
Enlarged vessels both sides

Intact nipple areolar complex.

Conclusion :

Small, sub-cm. Rt. upper lateral para-areolar Tu. Mass , with signs suggesting malignancy measures about 6mm. average diam.

Tri-cut guided needle biopsy is advised for further evaluation.

Many Thanks,,

Dr. M. Nafady, MD

Clin

Many Thanks,,
PROF. DR. M. NAFADY, MD
M. Nafady

Patient Name: السيد / ربيعة محمد إبراهيم

Date: 16 March, 2025

Referring physician: *Prof. Dr. HANY SEIF.*

TECHNIQUE:

Non-Enhanced Multiphase MSCT of the lumbar spine & the pelvis.

MORPHOLOGY:

- Marked osteopenia is evident. Innumerable confluent ill defined lytic lesions are involving the examined vertebral bodies.
- Reduced height of D11 and L2 vertebral bodies with posterior cortical erosion and anterior epidural compression at the lower level.
- Similar density lesions are involving the pelvic bones, the largest is involving the left iliac bone causing erosion of the anterior cortex yet with no extra-osseous soft tissue abnormality.
- Normal proximal femora.
- Bilaterally fractured pars interarticularis of L5 with forward slippage of L5 over S1.
- Normal alignment of the examined segments of the spine.
- No ligamentous hypertrophy or apophysal arthropathy.
- Normal features of the paraspinal tissues.

IMPRESSION:

Lytic lesions of the spine and pelvic bones, collapsed D11 and L2 vertebral bodies with anterior epidural compression at the lower level.

Sincerely,

Dr. Hazem Abu Zeid Yousef MD

Dr. **Elia Anis Ishak**

M. Sc. Ph. D. Pathology - Prof. of Pathology, Cairo University
Fellow, Armed Forces Institute of Pathology, USA
Member, United States Canadian Academy of Pathology

Dr. **Shady Elia Anis**

M. Sc. MD Pathology, Ass. Prof. of Pathology, Cairo University
Fellow, Mass General Hospital, Harvard, USA
Member, United States Canadian Academy of Pathology

Patient Name : رئيسة فهد البراهيم وزكري

Analysis ID :  939753

Age : 62 Y : 1 M : 1 D

Gender : Female

Referred By Prof. Dr. هاني سيف

Clinical Diagnosis : Left iliac bone osteolytic lesion in a case with multiple axial skeleton osteolytic lesions.

Nature of Specimen : Tru cut core biopsy.

Receiving Date : 2025-03-17

Patient ID : 699125

PATHOLOGY REPORT

Gross :

Multiple tan tissue cores, the longest measured 1 cm long, totally submitted.

Microscopic :

Serial sections examined from the specimen received revealed fibrofatty tissue and skeletal muscle infiltrated by round cell infiltrate with showing eccentric hyperchromatic nuclei.

Diagnosis :

Left iliac bone osteolytic lesion, Tru cut biopsy, MALIGNANT ROUND CELL TUMOR WITH PLASMACYTOID FEATURES.
- CASE FOR IMMUNOPHENOTYPING.

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak





This report is electronically signed, any alteration or modification would render it void.

تم التوقيع إلكترونياً - أي تعديل أو تغيير في النص
يؤدي إلى بطلان التقرير
01287005217 - 25790505 - 25773253 - 25744742 : هاتف

Dr.

Elia Anis Ishak

M. Sc. Ph. D. Pathology - Prof. of Pathology, Cairo University
Fellow, Armed Forces Institute of Pathology, USA
Member, United States Canadian Academy of Pathology

Dr.

Shady Elia Anis

M. Sc, MD Pathology, Ass. Prof. of Pathology, Cairo University
Fellow, Mass General Hospital, Harvard, USA
Member, United States Canadian Academy of Pathology

Patient Name : ربيعه فهمه ابوالاحم وزيري

Analysis ID : 941282

Age : 62 Y : 1 M : 2 D

Gender : Female

Referred By Prof. Dr.:

Clinical Diagnosis : Left iliac bone osteolytic lesion in a case with multiple axial skeleton osteolytic lesions.

Nature of Specimen : Block labelled as 939753 (Tru cut core biopsy)

Receiving Date : 2025-03-23

Patient ID : 699125

Technique :

Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP-DAB as chromogen and hematoxylin as counter-stain.
The sections were treated against:
CD 20 \ CD 138 \ Kappa \ Lambda

Results :

Tumor cells are NEGATIVE for CD 20.
Tumor cells are POSITIVE for CD 138.
Tumor cells are STRONGLY POSITIVE for Kappa.
Tumor cells are NEGATIVE for Lambda.

Conclusion :

Block, FINDINGS ARE COMPATIBLE WITH PLASMA CELL MYELOMA WITH KAPPA LIGHT CHAIN RESTRICTION.

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak

Shady Elia

Elia anis

This report is electronically signed, any alteration or modification would render it void.

عشرة رمسيس - بلاك أ - ميدان رمسيس - الدور الثالث و الثاني
01287006219 - 257400645 - 25773253 - 25744742 : تلفون



EL KHAWATIA HOSPITAL

منظمة الأطباء بـ
الأنشطة التشخيصية

C.T. US. DUPLEX. XRAY

Patient Name: Raesa Faheem

ProfDr:

21 April 2025

MSCT Scan of the Lumbo-Sacral Spine

- ↻ Straightening of the lumbar lordosis likely of muscle spasm.
- ↻ Few level hypodensity lytic lesion seen affection vertebral body more at D11 and L2 superimposed with reduced height and slight retro-bulge of L2. (picture suggestive of ?bony mets complicated with pathological fracture collapse of D11 and L2).
- ↻ Evidence of L5-S1 bilateral fracture pars articularis with L5-S1 mild diffuse bulge effecting theca.
- ↻ Advanced spondylodegenerative changes in term of mild reduced disc height, marginal osteophytes and facet joint athropathy.
- ↻ Intact posterior neural arches.
- ↻ Sagittal diameter of bony spinal canal is within normal.
- ↻ No paraspinal soft tissue lesions.

Signature

Dr. Mostafa.A.Ahmed

MSc

٩٣ / ٤٦٧٧٨٦٧ . ٤٦٧٧٨٦٧ . ٤٦٦٩٧٥٩ : ■

بـ . شارع الهر

MINISTRY OF HEALTH AND POPULATION
SPECIALIZED MEDICAL CENTER
SOUHAG CANCER CENTER



وزارة الصحة والسكان
مركز أورام سوهاج
مركز أورام سوهاج

Patient Name :
Patient No. : 26204012600626
Path No. :
Sex : Female
Age : 63 Y
Referring Doc : امل

Request No. : 110272939
Request Date : 17/11/2025
Verification Date : 17/11/2025
Financial Category : امل
Department : خارجي - سوهاج

Bone Marrow Report

Nature of specimen: Bone Marrow Core Biopsy
Clinical data:

Site: RT PSIS

Bone marrow biopsy is hypocellular.
Cellularity to fat ratio: Couldn't be assessed.
Erythropoiesis is decreased.
Granulopoiesis is decreased.
Lymphocytes are decreased.

Conclusion /Recommendation: Follow up of a case of MM with hypoplastic marrow , for CD138.

Signature

Dr/Noha Gaber

Printing Date : 17/11/2025 14:05:41

Page 1 of 1

Printing By : Mohamed Alaref
E-MAIL : AWRAM2017@GMAIL.COM

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SPECIALIZED MEDICAL CENTER
SOUHAG CANCER CENTER



وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مركز أورام سوهاج

Patient Name : رشيده فؤاد
Patient No. :
Path No. : 4609/2025m,
Sex : Female
Age : 45 Y
Referring Doc : 2

Request No. : 110277955
Request Date : 30/11/2025
Verification Date : 30/11/2025
Financial Category : أعلى
Department : خارجي - سوهاج

Pathology Report

IHC report

Clinical data:

Nature of specimen : BMB

Markers IHC for : CD138

Microscopic examination : Limited cellular area

CD138 : positive < 5%.

Signature
Dr. NohaGaber

Printing Date : 01/12/2025 12:47:45

Page 1 of 1

تاريخ الطباعة : 01/12/2025 12:47:45

Printing By : Dr / Reem Yahya

الطباعة بواسطة : د. ريم يحيى
E-MAIL : AWRAM2017@GMAIL.COM

Patient name: ربيعة فهد

Date: 2 February 2026

Abdominal US Report

- **Liver:** Average size. Normal echopattern. Smooth surface. No focal lesions. No intra-hepatic biliary channel dilatation or hepatic venous congestion.
- **PV:** Not dilated.
- **Spleen:** Not enlarged. Normal homogeneous echopattern. No focal lesions.
- **GB:** Average capacity. Thin wall. Clear bile content. No stones or masses
- **CBD:** Not dilated. No stones seen inside it.
- **Pancreas:** No abnormalities detected at its region.
- **Both kidneys:** Normal site, size and shape. Normal parenchymal tissue thickness. Normal echogenicity. No back pressure changes. No stones or masses.
- **UB:** Average capacity. Thin wall. Clear urine content. No stones or masses.
- **No ascites**
- **Diffuse colonic gaseous distention with preserved intestinal motility**
- **No evidence of inflammatory masses**

Conclusion:

>>> Diffuse colonic gaseous distention otherwise within normal pelvi-abdominal US findings

Much thanks for your kind referral,
Radiologist,
Dr ahmed akrab



ابيلنا - شارع مجلس المدينة - مقابل حلواني علوان

0110 446 446 1

Radio Scan



Request No. : 110309115
Request Date : 23/02/2026
Verification Date : 23/02/2026
Financial Category : Capital
Department : Public - Health

report

Clinical data LM

Site RT PSLs

concentrations of 10:10:80

body speculæ are normal

Stomatopoda is depressed

Lymphocytes are depressed

Megakaryocytes are decreased

Fibrosis is not present

Granuloma is not present

Conclusion /Recommendation: Follow up case of MM for CD138 Signature

Signature

Dr / Ahmed Makboul

Index

Printing By: Dr / Mub...

مدينة سوخاج الطبية - تلخون : ٠٤٣٣١١٠٩٠١ - ٠٤٣٣١١٠٩٠٢ - فاكس : ٠٤٣٣١١٠٩٠٣

E-MAIL: AYRAM2017@GMAIL.COM

MINISTRY OF HEALTH AND POPULATION
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وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مركز أورام سوهاج

Patient Name : السيد / ربيعة نعيم إبراهيم
Patient No. :
Path No. : 755/2026
Sex : Female
Age : 64 Y
Referring Doc : ...

Request No. : 110309283
Request Date : 23/02/2026
Verification Date : 28/02/2026
Financial Category : أهلي
Department : خارجي - سوهاج

Bone Marrow Report

IHC report

Clinical data: MM

Nature of specimen : BMB

Markers IHC for : CD138

Microscopic examination : About 5-10%

Signature
Ahmed makbool

Printing Date : 08/03/2026 13:25:51

Page 1 of 1

Printing By : Path.sohag

سوهاج : شارع كورنيش النيل الطبي مدينة سوهاج الطبية - تلفون : ٠٩٣٢١١٠٩٠١ : فاكس : ٠٩٣٢١١٠٩٠٣
E-MAIL : AWRAM2017@GMAIL.COM



مرکز رادیو سکن للاشعة ECHO-Doppler Report

Name : رئيسة فهم البراهيم Age : 64 years Date : 2/2/2026

Referred by: Prof. Dr.

Measurements

LVEDD	5.0	(3.5-5.6 cm)	Left Atrium	3.8	(1.9-4 cm)
LVEDD	3.2		Aortic Root	2.9	(2-3.7 cm)
IVSWT	0.6	(0.7-1.1 cm)	Ao. Cusp Separation		(1.6-2.6 cm)
LVPWT	0.7	(0.7-1.1 cm)	Ao. Annulus		
EF	64	> 55 %	Rt. Ventricle		(0.7-2.3 cm)

Two Dimensions and M-Mode Echocardiographic Findings

Mitral Valve : Sclerotic.		
Motion:	Calcification:	Thickness:
Score:	EPSS.....cm	Mitral area by 2D: Normal cm ²
The Aortic Valve: Normal.		
The Pulmonic Valve: Normal.		
The Tricuspid Valve: Normal.		
LV and Wall Motions: BBB motion of the septum.		
IV Septum: intact by TTE.		
IA Septum: Normal.		
Thrombi or Masses: No by TTE.		
Pericardium: Normal.		

Doppler and Colour Flow Findings

	Max./MN. PG	Valve area	Regurge/Grade	Jet area..cm ²
Mitral V.	mmHg	-	Moderate	-
Aortic V.	6 mmHg	-		-
Tricuspid V.	25 mmHg	-	Mild	-
Pulmonic V.	mmHg	-		-

Conclusion:

- Normal LV internal dimensions, good systolic function EF 64 %, SWMA at rest as described above, Diastolic dysfunction grade 1.
- Upper limit of normal LA, normal right side internal dimensions.
- Moderate MR, mild TR, PASP= 40 mmHg.
- Normal pericardium, no intracardiac masses by TTE.

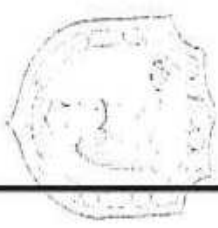


اربعاء ١٤٤٦ هـ الموافق ٢٠٢٤
المدينة - مقابل حلواني علوان

0110 446 4461

Radio Scan





Dr. Mohamed Elmaghrabi
Radiologist
Signature

Name: محمد عبد الحليم

Age: 65 Y

Date: 18-2-2026

Procedure:

AP of the skeletal whole body in the following sequences: coronal T1, Coronal T2WI, Axial T2.

Morphology:

A known case of multiple myeloma received CTII on follow up, in compared to the previous study dated on 11-3-2025

- The same as regard multiple focal bone marrow lesions displaying high signal in STIR, iso signal in T1 seen involving the lumbar spine and T12 bones with vertebral body collapsed of D11 and L2.
- No soft tissue masses
- Normal features of the paravertebral soft tissue structures.

Signature
Dr. Mohamed Elmaghrabi
MSc. MR
Radiologist
Refined by R/Mariam, Soohy